

Physiotherapy management plan

This management plan is intended to be a tool for you to review and discuss the progress of your treatment with your patient. This can be completed every 10 treatment sessions, or as requested by the case manager, and may be charged for using the PTMP code.

Patient details

Name:	Claim number:		
Working diagnosis/ symptoms:	☐ EML	☐ GB	☐ EnABLE Unit
Date of initial treatment:	No. of treatments to date:	(including exercise and group sessions)	

EVALUATION	Screening of risks for non-recovery										
	Psychosocial screening tool(s) used	Previous (☐ check if first form)		Update		Progress					
		Date	Score	Date	Score						
	Orebro Short Form (/100)					☐ Yes ☐ No					
						☐ Yes ☐ No					
	Impact of treatment (on functional abilities using validated functional outcome measurement questionnaires)										
PLANNING	Outcome measures used (recommend ≥ 2)	Previous (☐ check if first form)		Update		Progress					
		Date	Score	Date	Score						
						☐ Yes ☐ No					
						☐ Yes ☐ No					
						☐ Yes ☐ No					
ISSUES	SMART goals (short and long term)										
	Functional and work goals	Estimated date of achievement	Plan of how it will be achieved (E.g. treatment type, frequency and no.; self-directed program; etc.)								
	1.										
	2.										
	3.										
Estimated discharge date:											
YOUR DETAILS	Treating physiotherapist's details										
	Print name:	Practice name:									
Address:											
Phone:		Email:									
Signed:		Date:									
Clinical Framework for the Delivery of Health Services (2012) details best practice in achieving optimal outcomes for your patients. The signing physiotherapist agrees that the patient has been involved in completing this form and a copy provided to the patient, case manager and doctor. Plans can be submitted via Provider Online Services at www.rtwsa.com and select the 'Login' button, or via email to the relevant claims agent: <table border="0"> <tr> <td>EML</td> <td>Gallagher Bassett</td> <td>EnABLE Unit</td> </tr> <tr> <td>Email faxes@eml.rtwsa.com</td> <td>AHPlans@gb.rtwsa.com</td> <td>EnABLE@rtwsa.com</td> </tr> </table>						EML	Gallagher Bassett	EnABLE Unit	Email faxes@eml.rtwsa.com	AHPlans@gb.rtwsa.com	EnABLE@rtwsa.com
EML	Gallagher Bassett	EnABLE Unit									
Email faxes@eml.rtwsa.com	AHPlans@gb.rtwsa.com	EnABLE@rtwsa.com									

Physiotherapy: Functional ability recommendations

(Please complete the relevant section(s) below. Note: completing this page is optional)

Dear Dr _____

I have seen this patient for ____ sessions over the last ____ weeks for the above condition.

Following my clinical assessment, please find my recommendations of current functional abilities for this patient, which I suspect will be able to be further upgraded by _____ (date).

Physical function	Ability			Comments (frequency, duration, load, postures etc)
	Can do	Can do with modification	Cannot	
Sitting				
Standing / walking				
Kneeling / squatting				
Carrying / holding / lifting				
Reaching above shoulder				
Bending				
Use of affected body part				
Neck movement				
Climbing steps / stairs / ladders				
Driving				
Other				
Other				

☐ In my opinion, recovery will be assisted by incorporating work in their current program.

(Please check as appropriate)

Our management is focussed on

Signed: _____ Date: _____

Provider's name: _____

Physiotherapy management plan – explanatory guide

The Physiotherapy management plan should be completed where requested by the claims manager. The management plan can be initiated by the treating physiotherapist every **10 treatments** where this supports and facilitates treatment review and discussion with the worker and/or treatment team.

Please note: Physiotherapy management plans are practice-specific for individual patients (i.e. relevant to the treatment occurring at your practice).

Patient details	- Your working diagnosis may differ from the certifying doctor's injury description on the Work Capacity Certificate. This information will assist the claims management process by highlighting the need to obtain clarity around the diagnosis. - If your working diagnosis differs from that of the certifying doctor, it may be appropriate to have a verbal conversation in which the diagnosis or appropriate management can be clarified for consistency. - If treatment has been provided prior to this management plan, confirm the date of initial consultation and the total number of treatments provided to date. - A 'treatment' refers to all consultation types, i.e. initial, standard subsequent, long subsequent and restricted consultations; all individual and group aquatic and exercise physiotherapy sessions, and; workplace visits.
Evaluation	The evaluation process is an important part of the Clinical Framework for the Delivery of Health Services. Measurement and evaluation of treatment effectiveness helps to inform the treating team, patient and case manager on the direction and rate of change and to provide useful information to target treatment and enhance outcomes. If physiotherapy management has not achieved an improvement in function, it is anticipated that the physiotherapist will reassess the treatment approach and consider other strategies, which may include a workplace visit, case conference, referral for second opinion or ceasing treatment. Psychosocial screening (Principle 2 of Clinical framework) - Poor or delayed recovery can sometimes be explained by biological factors such as serious injuries or conditions, however psychosocial (non-injury) factors often contribute to poor recovery and return to work outcomes for patients. - Identification of risk factors early in the assessment phase should inform and guide the physiotherapist's treatment. - These factors should be screened at an early stage using psychosocial screening questionnaires, for example: - Orebro Musculoskeletal Pain Questionnaire Short or Long form (please state which version used) - Tampa Scale for Kinaesiophobia - Pain Catastrophisation Scale - Pain Self Efficacy Questionnaire. - Interviewing techniques and comprehensive history taking can also be used to identify risk factors. Psychosocial risk factors may include severe pain and disability reporting, low patient expectations about return to work, greater perceived treatment requirements, low job satisfaction, low self-efficacy, fears and anxieties or workplace matters. Outcome measures (Principle 1 of Clinical framework) - The outcome measure(s) used should be reliable, valid and sensitive to change. - They should relate to the patient's injury, the functional goals of treatment and the functional demands of the pre-injury duties. - Baseline measurement of the patient's functional capacity should be performed at the commencement of treatment and then serially re-measured to assess and demonstrate the benefit of treatment in improving functional capacity. - Physical impairment measures (such as ROM, numerically-rated pain scores, etc.) are clinically useful to track changes within a treatment session, however they do not reflect work capacity, activity limitations or participation restriction and therefore should not be used in isolation. - Achieving a change in outcome measure may be included as a goal if desired.
Planning	- Physiotherapy goals should relate to function and return to work. - Early and collaborative goal setting ensures that the patient and physiotherapist are focused on similar objectives to achieve recovery and return to work. - It is expected that the physiotherapist will gain an understanding of the patient's pre-injury role to ensure that the delivery of physiotherapy management is focused on early return to work and restoration to usual functional activities. - Collaborative goal setting empowers the patient to manage their recovery (Principle 3 of the Clinical Framework). - Goals should be specific, measurable, appropriate, relevant and timed (SMART) (Principle 4 of the Clinical framework). - Examples of SMART physiotherapy goals include: - To increase driving capacity to a one hour trip each way, with a short break as required, by 13 December - Increase work hours from 10 hours/ week to 20 hours/week by 31 December, with duties unchanged Plan of how it will be achieved - The physiotherapist should detail the treatment that is planned to be undertaken to achieve each goal. Your treatment should be based on the best available evidence (Principle 5 of the Clinical Framework).
Estimated discharge date	- This date is your estimation of when the patient will be ready for discharge and confident in their self-management, according to your current assessment and clinical reasoning. - It is understood that this date is not absolute and may move earlier or later depending on unforeseen circumstances.
Issues/Risks identified that may impact recovery and return to work	- If the patient has not made the expected progress, briefly outline why this might be the situation and recommended actions to address issues and overcome barriers. - The patient should be actively involved in a discussion of any factors that may increase the risk of non-recovery. The patient should be encouraged to raise any issues with the relevant parties to overcome any challenges to recovery and return to work. - If the physiotherapist's professional opinion is that the patient may not be able to return to their pre-injury duties, please indicate in this section and detail the factors that lead to this opinion. The physiotherapist's early identification and communication of such matters is very important to ensure that the end goal of rehabilitation is appropriate for the patient.

- Completion of this page is optional. Your clinical judgement can provide important recommendations to the treating team, as your opinion of the patient's current abilities will facilitate the identification of appropriate duties.

ReturnToWorkSA
Enquiries: 08 8238 5757
400 King William Street, Adelaide SA 5000
providers@rtwsa.com
www.rtwsa.com

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